

Workday Cost Center Governance & Analytics

A controllership-grade control framework over Workday HCM & Financials extracts — exception triage, budget variance, and labor analytics for a regional health system.

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Python · SQL-style analytics · Streamlit · Workday concepts (EIB, supervisory orgs, worktags, journals)

Executive summary

Cost centers are the foundation every healthcare finance number stands on. When the cost-center master is wrong — orphan codes, lapsed owner attestations, journals that bypass segregation of duties — budgets misallocate, labor spend hides, and the month-end close slows. I built an end-to-end governance program that treats Workday extracts the way a controllership team should: **validate first, analyze second**.

22	Active cost centers governed
55	Exceptions surfaced (28 critical)
7	SOX-style controls, severity-ranked with SLA triage
90%	Owner attestation compliance (Q2 FY25)
72.8%	Journal auto-post rate
\$+0.3M	Net TTM budget variance on \$429M spend

Bottom line: the framework caught spend posting to a closed cost center, 25 segregation-of-duties violations, and an agency-labor spiral in nursing that is driving unfavorable variance — the exact findings a Finance leader needs before close, not after.

Why cost-center governance decides close quality

- Every journal, payroll run, and budget load in Workday keys off the cost center. Bad master data compounds into every downstream report.
- In healthcare, labor is 55–60% of operating expense. If agency and overtime dollars land on the wrong codes, productivity metrics lie.
- Auditors test exactly these controls: SoD on journals, approval thresholds, and attestation evidence. Findings here are audit findings.

Control framework

Seven preventive/detective controls, each mapped to a Workday concept and ranked by financial-statement risk. Critical findings carry a 5-day remediation SLA; High, 15 days.

Control	Severity	Control name	What it checks
CC-01	Critical	No orphan cost centers	Every cost center maps to an active supervisory org
CC-02	High	Owner attestation current	Quarterly attestation; no lapse in last 2 quarters
CC-03	Critical	Segregation of duties	Journal requester ≠ approver
CC-04	High	Large-journal approval	Journals > \$25K require director approval

Control	Severity	Control name	What it checks
CC-05	Critical	No spend on inactive codes	Closed cost centers must carry zero spend
CC-06	Medium	Worker cost-center assignment	Every active worker assigned to a cost center
CC-07	Medium	Budget coverage	Every active cost center has an FY budget

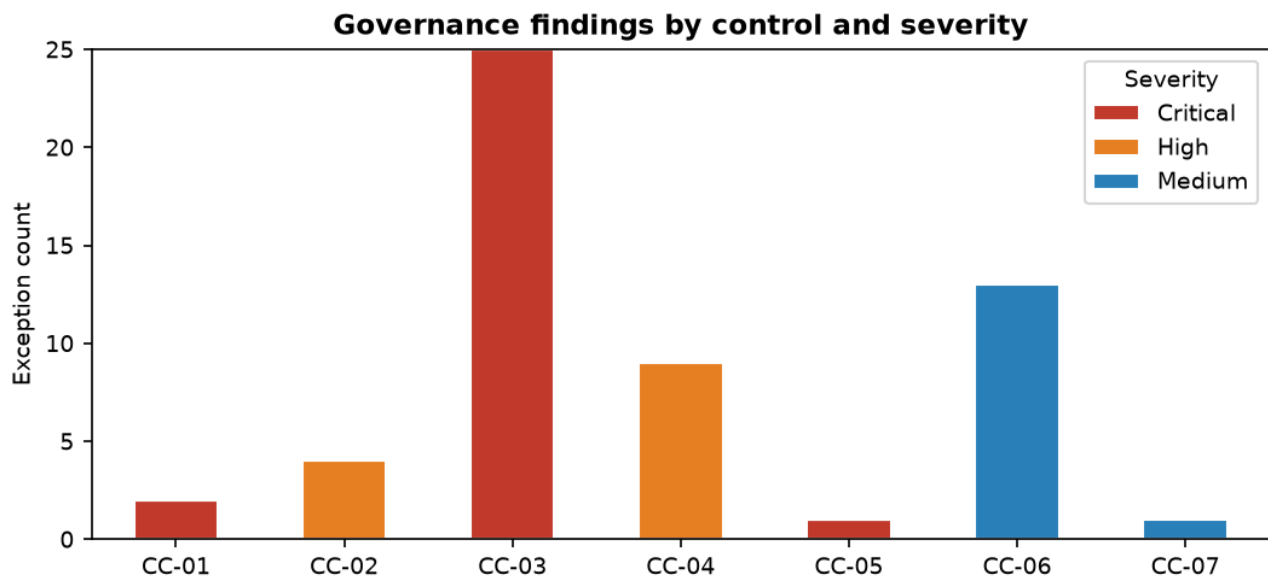


Figure 1 — Findings by control. Journal SoD (CC-03) is the weakest control; master-data defects (CC-01/CC-05) are few but critical.

What the data found

The money story: premium-labor spiral in nursing

Med/Surg Nursing (CC-1080) and ICU (CC-1090) are the only cost centers running over budget — +\$1.06M (+2.9%) and +\$1.04M (+3.5%) respectively. The driver is not base wages; it is agency labor, which has climbed past 10% of base pay in both units while overtime holds at 7% system-wide. Agency dollars cost roughly 2× employed staff, so a double-digit agency mix mathematically guarantees unfavorable variance. Recommended actions: float-pool expansion, targeted retention review, and a 90-day agency-reduction target with weekly tracking — the standard playbook, now backed by the numbers.

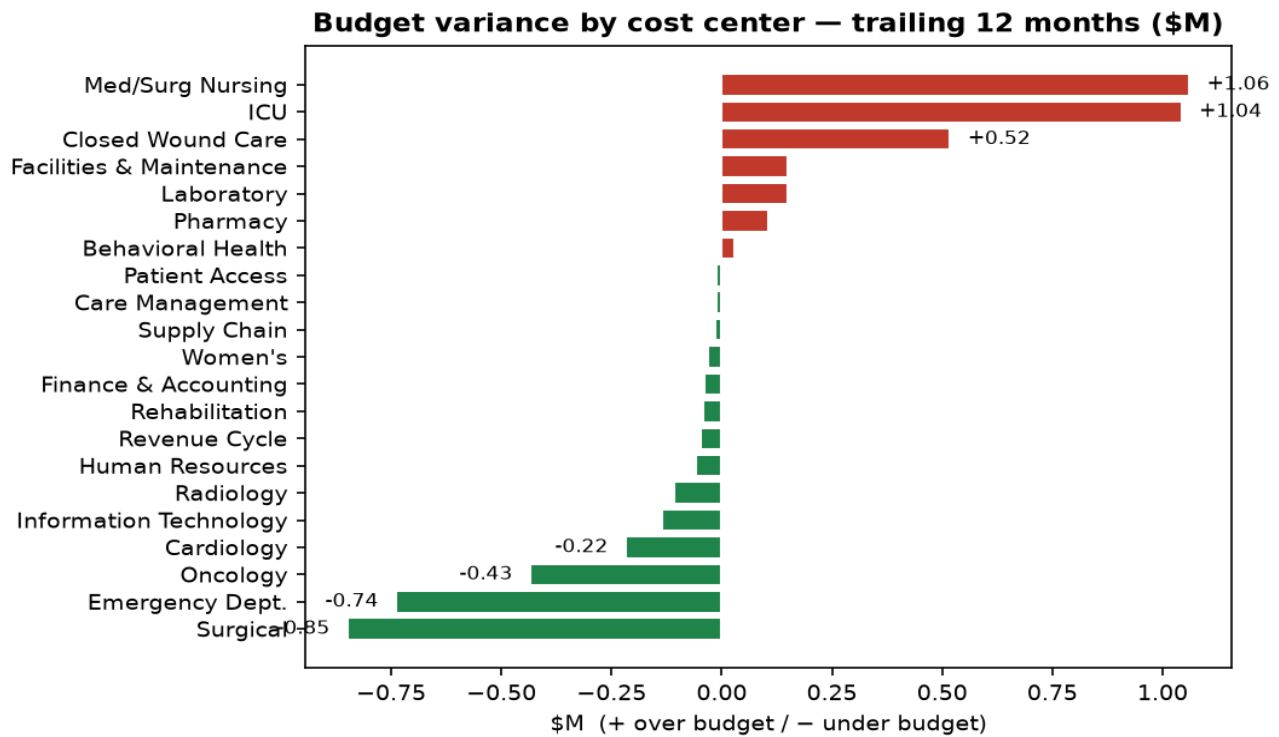


Figure 2 — Trailing-12-month budget variance by cost center.

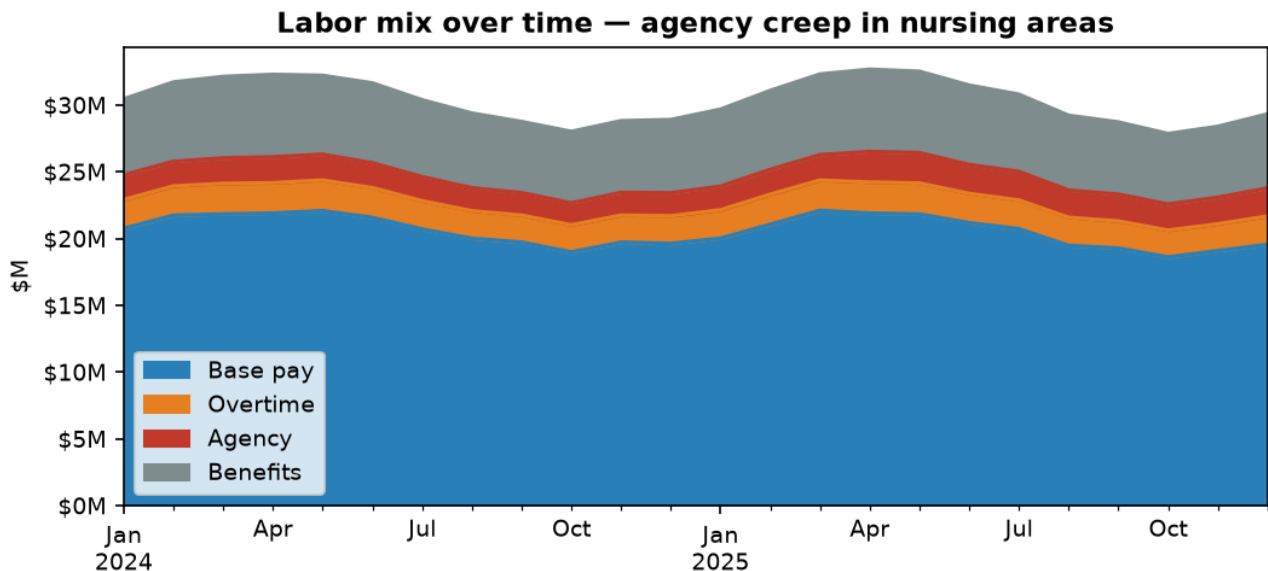


Figure 3 — Labor mix trend. The agency (red) band widens in the last two quarters — the visual signature of the spiral.

Master-data and journal findings

- **\$518K posted to a closed cost center (CC-3010, Closed Wound Care).** Spend hitting an inactive code means budgets and productivity metrics for the receiving units are wrong. Fix: hard-stop validation on the EIB payroll load.
- **25 journals where requester = approver.** A textbook SoD failure and an audit finding waiting to happen. Fix: Workday business-process routing that physically prevents self-approval.

- **9 journals over \$25K without director approval**, plus attestation compliance at 90% with Operations the weakest division — the attestation process needs an owner escalation workflow, not another reminder email.

How I'd run this in production

- **Monthly EIB extracts** from Workday HCM/Financials into the control engine; exceptions auto-routed to cost-center owners with SLA clocks.
- **Pre-close checkpoint:** CC-01 through CC-05 must clear before the close checklist opens — no more discovering master-data breaks on day 4 of close.
- **Quarterly attestation campaign** with escalation to division finance leads at day 10, not day 30.
- **Executive read-out:** one page — exceptions by severity, variance vs. budget, agency/overtime trend. The dashboard in this repo is that page, live.

Build notes

Python (pandas, NumPy) for the extract simulation and control engine; a severity-ranked exception queue as the triage artifact; Plotly/Streamlit for the command-center dashboard; matplotlib + ReportLab for this document. All data is synthetic and modeled on real Workday concepts — supervisory orgs, worktags, ledger accounts, EIB loads, and business-process approvals. Repository includes the full pipeline: [generate](#) → [govern](#) → [analyze](#) → [visualize](#).

About the author

Sundara Prasanthi Movva is a Senior Financial Analyst in healthcare FP&A — Financial Analyst at Humana (2025–present), Senior Financial Analyst at Optum / UnitedHealth Group (2021–2022), Financial Analyst at Cigna (2017–2021) — with an MBA in Business Analytics. Day-to-day work: budgeting, variance analysis, labor-spend analytics, and Workday cost-center governance across 40+ cost centers.